10.7 Disseminated Intravascular Coagulation (DIC)*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective *No specific complaint* **Objective** • Oozing from IV and puncture sites • Hematuria • Low platelets and fibrinogen; raised D-dimer • BP 92/58 mmHg	**ACTUAL** Inadequate fluid volume related to active bleeding from coagulopathy as evidenced by oozing, hematuria and falling BP.	• Less bleeding and stable vital signs within 24 hours.	• Support treatment of the cause: sepsis, obstetric complication or trauma. • Give platelets, plasma and cryoprecipitate as prescribed. • Hold pressure on puncture sites for longer than usual. • Follow vital signs, hemoglobin and clotting results. • Avoid needless punctures.	• Considered met when bleeding lessens and vital signs settle.
Subjective *No specific complaint* **Objective** • Cool, mottled extremities • Slow capillary refill	**ACTUAL** Ineffective peripheral tissue perfusion related to microvascular thrombosis as evidenced by mottled, cool extremities.	• Warm extremities; capillary refill under 3 seconds.	• Check the extremities hourly. • Keep her warm. • Report cyanosis or signs of new organ failure.	• Goal reached as perfusion improves.
Subjective *No specific complaint* **Objective** • Coagulopathy continuing • Sepsis	**RISK** Risk for shock as evidenced by risk factors: hemorrhage and sepsis.	• MAP above 65 mmHg; urine at least 0.5 mL/kg/hour.	• Follow hemodynamics and hourly urine output. • Give fluids and blood products as ordered.	• Met while she remains stable.

*References for this care plan: [20], [35], [70], [128]. Full entries are listed in the References section of the CPMN book.